

Oxford BioMedica plc **oxb**

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by

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			2026	2027
Price:	4.87	EPS	0	0
Shares Out. (in M):	121	P/E	0	0
Market Cap (in \$M):	589	P/FCF	0	0
Net Debt (in \$M):	-21	EBIT	0	0
TEV (in \$M):	568	TEV/EBIT	0	0

Description

I first wrote up OXB a year ago. It's up 37% from there after a wild swing upwards on a takeover attempt that was ultimately rejected and now a steep fall from credibility issues related to a poor / missed management guide.

I'm writing up OXB again because it remains my highest conviction position at this price with the launch of its main supplied therapy, Gilead's anito-cel, about to play out.

EQT bid 930p for OXB in February 2026, and the board rejected it as undervalued, that is roughly 90% above today's 490p. While not a floor, it gives you a sense of what an informed buyer thought the asset was worth and the board / major shareholders even higher estimation of worth.

Anito-cel will be the single biggest driver of OXB's value over the next three years and has a December 23, 2026 PDUFA date (very high likelihood of approval). Anito-cel was developed by Arcellx which Gilead paid roughly \$7.8bn to own outright in February. OXB supplies the lentiviral vector to anito-cel.

It will be the third Car-t to enter the multiple myeloma (blood cancer) space, usually not a good spot. But so far it's had similar to better data as that of its main competitor Legend's Carvykti (cilta-cel) without the tox issues and much better data than BMS's Abecma (ide-cel).

Clinicians expect it to take share fast, starting from the fourth line it will be approved in. The main issues with Carvykti as reported by doctors:

- Delayed neurotoxicity. Carvykti in particular carries a risk of Parkinsonism, cranial-nerve palsies and Guillain-Barré-type syndromes that can appear weeks to months after infusion. Across anito-cel's trials to date, clinicians reported none of these delayed neurological events.
- Immune-effector-cell enterocolitis. A newer, poorly understood complication seen with cilta-cel, severe gut inflammation carrying roughly a 40% mortality rate when it occurs. Anito-cel has not shown it.
- Low CRS/ICANS. About 95% of anito-cel patients had grade 1 or no cytokine-release syndrome, and essentially no high-grade neurotoxicity, which is what makes outpatient dosing and community adoption realistic.

These were expressed by multiple myeloma KOLs (MD Anderson, Mass General, UT Southwestern) at Arcellx's December 2025 ASH investor event and have been backed up by outside MM experts who have expressed similar reservations about cilta-cel. At ASH one of the doctors indicated they were considering having some patients wait for anito-cel approval.

The company attributes the difference to the binder: anito-cel uses a compact D-domain BCMA binder rather than the antibody-derived binders in cilta-cel and ide-cel.

Anito-cel flow-through to OXB

I model anito-cel patient volumes, apply OXB's share of vector supply, along with an estimated price per dose. Three cases below. Bear runs below Carvykti's actual launch pace, never reaching its year 3-4 peak. Base is anchored to Carvykti's actual dose ramp, the best real-world CAR-T analog, with anito-cel modestly beating it (manufacturing now in the hands of Kite, which has pioneered Car-t manufacturing and done 24k doses across its other therapies. They claim they will be able to beat Legend/J&J manufacturing times at launch).

Bull reverse-engineers Arcellx's own management projections from the Gilead tender offer (the 14D-9), which imply a much faster ramp, but requiring anito-cel to beat every demonstrated CAR-T launch and are probably unlikely. All three assume OXB is sole vector source at launch (consistent with contracts we can see and Kite/Gilead commentary), holding majority share for years as Kite only gradually brings internal lentiviral vector capacity online (model assumea Kite reaches ~10% by 2030, ramping slowly from there so it becomes dual-sourced production).

Global anito-cel patients (estimated)

Case	2027E	2028E	2029E	2030E	2031E	2032E
Bear (sub-Carvykti)	500	1,200	2,000	3,000	4,000	5,000
Base (Carvykti-anchored)	1,000	2,000	3,500	5,000	6,500	8,000
Bull (14D-9 mgmt)	3,500	8,000	10,000	11,000	11,500	12,000

To note, Kite is working on building lentiviral vector manufacturing. J&J-Legend shows how hard this will be for Kite to complete quickly. J&J started building

LVV capacity when it signed the Legend collaboration in December 2017. But even as of launch in 2022 LVV manufacturing was a constraint and remains a constraint. Legend brought Novartis in as a second LVV maker in 2024.

It will be even harder for Kite. J&J was arguably the best-positioned pharma in the world to do this: it already had Janssen vaccines infrastructure, made the J&J COVID vaccine, and had decades of viral manufacturing experience. Kite has retroviral manufacturing experience. Unlike Legend/J&J, OXB has been making the vector used in all of the anito-cel trials so far, meaning any onboarding of new LVV maker Kite will require an extensive FDA process. They will need to file a Prior Approval Supplement backed by analytical and functional comparability data on multiple commercial-scale lots of both vector and finished CAR-T product. That process typically takes 12-18 months of side-by-side production before Kite can even file, then 4-6 months of FDA review, and it could need another 1-2 years if the comparability data isn't clean enough and clinical bridging is required, ie it's possible they would need new clinical data using Kite vector for anito-cel.

The realistic first-approved-Kite-vector on anito-cel is 2031, and dual-source with OXB continues in parallel for years after that.

OXB vector-supply assumptions

Assumption	2027E	2028E	2029E	2030E	2031E	2032E
OXB share of vector	100%	100%	100%	90%	88%	85%
Net price / dose (£k)	44	43	42	41	40	39

Net price per patient dose to OXB is estimated from management commentary, broker research, Carvykti gross margins.

Anito-cel revenue to OXB (£m)

Case	2027E	2028E	2029E	2030E	2031E	2032E
Bear	22	52	84	111	141	166
Base	44	86	147	185	229	265
Bull	154	344	420	406	405	398

Flow through to OXB total revenue / EBITDA

Revenue build on anito base case

£m	2024A	2025A	2026E	2027E	2028E	2029E	2030E	2031E	2032E
Anito-cel (Arcellx/Kite)				44	86	147	184.5	228.8	265.2
BMS (arlo-cel + pipeline clinical)	3	5	5	7	28	65.5	96.4	112.4	112
Other commercial	10	8	14	22	38	66	109	161	223
Base business (PD + clinical + P&S)	114	160	171	188	216	248	278	306	330
Total revenue (£m)	127	173	190	261	368	526.5	667.9	808.2	930.2
<i>management guidance path</i>			190	242	315	405	500	560	620
EBITDA margin %	(11.8%)	1.4%	5.0%	13.0%	20.0%	26.0%	30.0%	33.0%	35.0%
Operating EBITDA (£m)	(15)	2.4	9.5	33.9	73.6	136.9	200.4	266.7	325.6

Valuation summary

Metric	Value
Current share price (p)	490
Shares outstanding (m)	121
Market cap (£m)	£593
Net cash (£m)	£21
Enterprise value (£m)	£572
EQT rejected bid (Feb 2026, p)	930 (+89.8% vs current)

Implied value, 2028E EBITDA multiples (2028E EBITDA £73.6m)

Multiple	EV (£m)	Equity (£m)	Per share (p)	vs current
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Multiple	EV (£m)	Equity (£m)	Per share (p)	vs current
12.0x	£883	£904	747	+52.5%
15.0x	£1,104	£1,125	930	+89.7%
18.0x	£1,325	£1,346	1,112	+127.0%
20.0x	£1,472	£1,493	1,234	+151.8%
22.0x	£1,619	£1,640	1,356	+176.6%
25.0x	£1,840	£1,861	1,538	+213.9%

Implied value, 2030E EBITDA multiples (2030E EBITDA £200.4m)

Multiple	EV (£m)	Equity (£m)	Per share (p)	vs current
8.0x	£1,603	£1,624	1,342	+173.9%
10.0x	£2,004	£2,025	1,673	+241.5%
12.0x	£2,404	£2,425	2,004	+309.1%
15.0x	£3,006	£3,027	2,501	+410.5%
18.0x	£3,607	£3,628	2,998	+511.9%
20.0x	£4,007	£4,028	3,329	+579.4%

What it's worth

At 490p, OXB has a market cap of roughly £593m and an enterprise value near £572m (net cash £21m after the H1 draw). On the base case that is about 8x 2028E EBITDA and roughly 1.5x 2028E revenue. That is too cheap for a viral-vector CDMO with this growth and multiple clients nearing commercial launch.

Quality CDMOs change hands at 5x-plus revenue and 15x-plus EBITDA (refer to last writeup for comps); OXB is the dominant lentiviral vector manufacturer with a leading CAR-T about to launch on its vector.

- On Base 2028E EBITDA of ~£74m, a 15x multiple implies roughly 930p (+90% vs 490p); even a conservative 12x is ~745p (+52%).
- On Base 2030E EBITDA of ~£200m, 10x is ~1,675p.

The asymmetry that made this attractive last year is wider now that the stock has fallen and we've gotten solid anito-cel data in Dec 2025. Biosecure Act has come in, making any chance that underpriced Chinese manufacturing of VV unlikely to impossible. EQT bid provides a takeover floor.

EQT rejected at 930p

From press reports: EQT bid 930p for OXB in February 2026 and was turned down by the board / major shareholders as undervaluing the company.

EQT is still looking at CDMOs. It went deep in the auction for PolyPeptide, the Swiss peptide CDMO, and was shortlisted into the final rounds before losing to Samsung Biologics, which paid \$1.8bn in July 2026.

If they want to return to OXB, EQT should be able to bid around end August, based on lapse in UK takeover rules six month rule.

Why is OXB cheap again? Management credibility in question

On August 7 OXB cut FY26 revenue guidance from £220-240m to £180-200m, and took EBITDA margin guidance down to low-single-digit (reported). That is a real dent for a team that had reiterated the higher number as recently as the June capital markets day.

Based on explanations given by mgmt and RBC (their in-house broker), here is what happened:

- The anito-cel approval-pathway changed. OXB appears to have carried a forecast assuming an earlier or accelerated approval for anito-cel that assumed H2 launch, driving bigger 2026 launch-prep orders and scaling commercial volumes in H2. The BLA was accepted in Feb with Dec 2026 PDUFA date given then. Why mgmt didn't factor that in to their guide earlier is unclear and real ding to credibility.
- Durham site delay (an execution stumble, now mending). They bought Durham plant last year, but it's taken longer to get up to speed than originally estimated, early batch failures and a recent site-head replacement. Now recovering, GMP run underway as of Aug 7. Not unexpected when you integrate a new site, and Durham itself was a bargain (below).
- Smaller work packages, one early-stage cancellation. Softer near-term conversion; framed as timing, not lost programmes, and consistent with a record BD intake in 1H, a record 17 new client wins in H1 (19 by August, more than 30% above the whole of FY25).

---Durham: Durham was a genuine good deal, ex-Bluebird capacity acquired cheaply that gives OXB US commercial-scale supply for a fraction of build cost. Purchase positioned OXB for win of BMS Car-t programs which came in Feb.---

Other programs near launch:

Kyverna.

Kyverna's miv-cel (mivocabtagene autoleucel, formerly KYV-101) is a CD19 CAR-T aimed at autoimmune disease rather than cancer, the first serious attempt to take CAR-T into that far larger market. OXB signed a non-exclusive, multi-year license and supply agreement with Kyverna in September 2023 covering lentiviral vector for both clinical and commercial use across any Kyverna product, so OXB supplies the vector here just as it does for anito-cel. Kyverna is furthest along in stiff person syndrome (SPS), an orphan neurological indication, with a rolling BLA completing Q4 2026 and first approval likely in 2027. SPS is small indication, so the near-term revenue is modest. Bigger indications sit behind it like generalized myasthenia gravis (a registrational trial is ongoing) and then lupus nephritis. If CD19 CAR-T works in autoimmune the way early data suggest, miv-cel is OXB's optionality on the whole autoimmune-CAR-T thesis, not just a single orphan drug.

Arlo-cel. BMS's arlocabtagene autoleucel (BMS-986393) targets GPRC5D in MM, not BCMA like the other three (Carvykti, Abecma and anito-cel). Because arlo-cel hits a different antigen, patients who relapse after a BCMA therapy have no cross-resistance to it, which will give it a market. The Phase 1 data are strong. Realistic launch is 2028+. OXB's exposure runs through the BMS 5-year commercial supply agreement signed in February 2026.

Key risks

- With anito-cel in 2L trial results in 2027 and ramping commercial volumes, growing number of patients on therapy, so greater chance tox issues show up and the whole case for it over Carvykti erodes.
- Manufacturing throughput, not demand, has been the key limiting factor for autologous CAR-T's so far, even Carvykti took 3-4 years to reach ~5,000-6,000 annual doses. If anito-cel only matches rather than beats that curve, base volumes are too high.
- Faster-than-modelled Kite internal LV insourcing.
- Durham execution doesn't stabilise; further site or work-package slippage.
- Longer-term: LNP / non-viral delivery erodes the vector franchise.

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Catalyst

Anito-cel approval and commercial ramp

Messages

Subject	Couple questions
Entry	08/18/2026 06:17 PM
Member	ATM

Thanks for the write-up. Couple questions:

1. When you are thinking about the value here over time, what share count are you assuming by the time they get to 2030 - these folks have issued a lot of shares over the last five years? I also don't trust their view of capex or working capital as they have already shown an inability to forecast well. So this makes me want to assume a lot more shares outstanding because they might be successful, but it will be with more shares than today.
2. How much capacity have they built and what are the likely capex requirements to get to 2030? In traditional CMDOs, the capex costs are HUGE and there are further capital investments as you win customers. Often the customers have specific things (equipment) that they need you to have to run their process. Sometimes you can get the client to pay and often you can't.
3. They have made some assertion that their business model growth is accretive to working capital. This seems unlikely from my experience. Look at the larger peers and how much working capital gets stranded on balance sheets as these businesses grow. In many commercial contracts there are minimum inventory levels that have to be maintained in both finished product and components to go into manufacturing.
4. You reference the credibility issue of having the capital markets day and then cutting guidance right after. How do you get comfortable about a screw up like this? It totally undermines the credibility they were working to build with the capital markets day.

Thanks